

CLINICAL STUDY REPORT

FULL

<INTERIM <number>/FINAL>

If more than one interim report is planned, designate the number of the report (e.g., Interim 1, Interim 2). Also apply this number in the running headers. If only one interim analysis is planned, only designate “Interim.”

Study Number: <Number>

Study Title: <Title of protocol>

Name of Study Treatment: List generic name first (if available). Include trade name(s), if available and development name in parentheses. Use generic name (if available) or development name (if a generic name is not assigned) throughout document

Indication: <>

Development Phase: <1, 2, 3, 4>

Date of First Treatment: <dd month yyyy>

<End of Study Date: <dd month yyyy see protocol for definition>

<Date of Data Cut-Off: <dd month yyyy include for ongoing studies>

<Date of Early Study Termination: <dd month yyyy include if study terminated early>

Sponsor:.

Name of Sponsor Signatory: <Name>
<Title>

.

Sponsor’s Study Medical Director: <Name>
<Title>

If the study used a contract Study Medical Director, include here.

Report Date: <dd month yyyy>

Version: <X>

This study was conducted in accordance with the ethical principles of Good Clinical Practice,
according to the ICH Harmonised Tripartite Guideline.

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GENERAL TEXT INSTRUCTIONS FOR AUTHORS

Some guidance text is repeated at the beginning of Sections 10 and 11 in the case that different writers are assigned to different sections of the CSR.

The instructions below will not be repeated throughout the text.

Guidance text: Guidance text will appear as red hidden text and will describe what should be addressed in the section. When replacing guidance text, you must make sure to change the format style to C-Body Text or as appropriate. Otherwise, it will remain as hidden text. Ensure that hidden text is removed prior to finalizing the document.

Sample text: Sample text will appear as C-Body Text in <brackets> and may contain examples or choices that can be modified to fit the study, or instructions or placeholders for sentences where red hidden text is not applicable. Ensure that unused sample text is deleted prior to document distribution.

Standard text: Standard APPROVED text will appear as gray highlighted C-Body Text and should not be modified unless permission is granted from the appropriate function (e.g., Safety). Ensure that the gray highlighting is removed prior to document distribution.

Terminology: “participant” vs. “subject”: Per the Style Guide, the term “participant” is used instead of “subject” in text and in hand-tabbed (nonprogrammed) tables to refer to individuals who are taking part in clinical studies. Programmed tables (TLGs), however, may continue to use “subject.” (The same is true in CSRs.) The two terms are equivalent in meaning.

Heading levels:

Unless specifically stated in the relevant section of the template, do not delete Level 1 headings (e.g., Section 5). If a Level 1 heading is not relevant to the study, insert the statement, “Not applicable.”

The order of Level 1 headings SHOULD NOT be changed.

Level 2 headings and below (e.g., Section 4.3, Section 11.2.4) can be modified to reflect section content and may be deleted if the section is not relevant to the study. Do not indicate “Not applicable”; simply delete the heading. (Per the Style Guide, headings below Level 4 are discouraged.)

Table formatting:

In-text tables that are formatted *manually* (vs. using the StartingPoint table formatting tool) should use the following format:

- Table text (body): Arial 10 point, regular.
- Column headings: Arial 10 point, bold.
- Table rules: Horizontal only, for separation of rows; avoid use of vertical rules.
- Table footnotes: Arial 9 point, regular.
- Captions should remain New Times Roman, 12 point, bold.

See [Table 2](#) for an example of the preferred table format.

Cross-references: Use hyperlinks for all internal cross-references. For external cross-references, highlight all cross-references in blue text according to the instructions in the Cross-Referencing Guide for Global Regulatory Submissions.

TABLE OF CONTENTS

TABLE OF CONTENTS.....	5
LIST OF TABLES	9
LIST OF FIGURES	10
4. LIST OF ABBREVIATIONS AND DEFINITIONS OF TERMS.....	11
5. ETHICS	12
5.1. Independent Ethics Committee and/or Institutional Review Board	12
5.2. Ethical Conduct of the Study	12
5.3. Participant Information and Consent	12
6. INVESTIGATORS AND STUDY ADMINISTRATIVE STRUCTURE	13
6.1. Investigators.....	13
6.2. Study Committees.....	13
6.3. Vendors.....	13
6.4. Authors of the Clinical Study Report	13
7. INTRODUCTION	15
8. STUDY OBJECTIVES, <ENDPOINTS> <AND/OR> <ESTIMANDS>	16
9. INVESTIGATIONAL PLAN.....	18
9.1. Overview of Study Design.....	18
9.2. Discussion of Study Design.....	20
9.2.1. Changes to the Study Conduct.....	20
9.2.2. General Trial Conduct Mitigation Strategies During Public Health Emergencies.....	21
9.3. Study Population.....	21
9.4. Treatments	22
9.5. Study Activities and Assessments: <Efficacy, Pharmacokinetics, Pharmacodynamics, Biomarker, Safety, Immunogenicity>	24
9.6. Data Quality Assurance	24
9.6.1. Quality Risk Assessment	24
9.6.2. Key Quality Indicator Monitoring	25
9.6.3. Data Collection	25
9.6.4. Data Management and Quality Control	26
9.6.5. Audits.....	27
9.7. Statistical Methods Planned in the Protocol	27

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9.7.1.	Statistical Analysis Plan	27
9.7.2.	Changes to the Planned Analyses <Prior to Unblinding or Database Lock>	29
9.7.2.1.	Public Health Emergency-Related Changes to the Statistical Analyses	29
9.7.3.	<Changes Following Study Unblinding/Database Lock and Post Hoc Analyses>	30
10.	STUDY PARTICIPANTS	31
10.1.	Disposition of Participants.....	31
10.2.	Study Populations Analyzed.....	36
10.3.	Demographics	36
10.4.	Baseline Disease Characteristics	37
10.5.	Baseline Medical History	37
10.6.	Prior and Concomitant Treatments	37
10.6.1.	Prior <Current Indication> Treatments	37
10.6.2.	Concomitant <Current Indication>Treatments.....	37
10.7.	Protocol Deviations	38
10.7.1.	Serious Breaches of Good Clinical Practice	38
10.7.2.	Unblinding Events	39
10.7.2.1.	In the Case of a Medical Emergency	39
10.7.2.2.	Accidental Unblinding.....	39
10.8.	Exposure and Study Treatment Compliance	40
10.8.1.	Exposure	40
10.8.2.	<Dose Modification>	40
10.8.3.	<Compliance>	40
11.	EFFICACY <, PHARMACOKINETIC, PHARMACODYNAMIC AND/OR BIOMARKER> RESULTS	41
11.1.	Primary Efficacy Endpoint: <name of endpoint measure>	44
11.2.	Secondary Efficacy Endpoint: <name of endpoint measure>	45
11.3.	<Exploratory Efficacy Endpoints>	45
11.4.	<Pharmacokinetics>	45
11.4.1.	Study Treatment Dose, Concentration, and Relationship to Response	45
11.4.2.	Drug-Drug and Drug-Disease Interactions	46
11.5.	<Pharmacodynamics>	46
11.6.	<Biomarkers>	46

11.7.	Pharmacokinetic/Pharmacodynamic Analyses	46
11.8.	Post Hoc Analyses	46
11.9.	Statistical and Analytical Considerations in This Study.....	46
11.9.1.	Appropriateness of Model	47
11.9.2.	Adjustment of Covariates	47
11.9.3.	Handling of Dropouts and Missing Data	47
11.9.4.	Interim Analyses and Data Monitoring	48
11.9.5.	Multicenter Studies	48
11.9.6.	Multiple Comparison/Multiplicity	48
11.9.7.	Use of an “Efficacy Subset” of Participants	49
11.9.8.	Active-Control Studies to Show Equivalence	49
11.9.9.	Subgroup Analyses	49
11.10.	Efficacy, Pharmacokinetic, Pharmacodynamic, and Biomarker Conclusions	49
12.	SAFETY RESULTS.....	50
12.1.	Deaths, Serious Adverse Events, and Other Significant Events.....	53
12.1.1.	Deaths	53
12.1.2.	Serious Adverse Events	53
12.1.3.	Adverse Events of Special Interest	53
12.1.4.	Discontinuation of Study Treatment Due to Adverse Events.....	54
12.1.5.	Withdrawal From Study Due to Adverse Events	54
12.1.6.	Dose Modifications Due to Adverse Events.....	54
12.1.7.	Malignancies.....	54
12.1.8.	Pregnancies	54
12.1.9.	<Other Significant Events>	54
12.2.	Adverse Events	55
12.2.1.	Overall Summary of Adverse Events	55
12.2.2.	Public Health Emergency-Related Adverse Events	56
12.2.3.	Analysis by Severity	56
12.2.4.	Analysis by Relationship to Study Treatment	56
12.2.5.	Other AEs of Note	56
12.3.	Participant Safety Narratives	56
12.4.	Clinical Laboratory Results	57

12.4.1.	Hematology Results.....	57
12.4.2.	Blood Chemistry Results	57
12.4.3.	Urinalysis Results	57
12.5.	Vital Sign Measurements, Physical Examination Findings, and Other Observations Related to Safety.....	58
12.5.1.	Vital Sign Measurements.....	58
12.5.2.	ECG Findings	58
12.5.3.	Physical Examination Findings	58
12.5.4.	<Study-Specific Observations>	58
12.6.	<Immunological Assessments>	58
12.7.	Columbia Suicide Severity Rating Scale.....	59
12.8.	<Study-Specific Subgroups>	59
12.9.	Safety Conclusions	59
13.	CONCLUSIONS	60
14.	TABLES, FIGURES, AND GRAPHS REFERENCED, BUT NOT INCLUDED IN THE TEXT.....	61
14.1.	Study Participants	61
14.2.	Efficacy, Pharmacokinetic, Pharmacodynamic, and Biomarker Data.....	62
14.3.	Safety Data.....	63
14.3.1.	Narratives of Deaths, Serious Adverse Events, and Other Significant Adverse Events	63
14.3.2.	Summary of Adverse Events	63
14.3.3.	Summary of Clinical Laboratory Results	63
14.4.	Additional Information on Study Design and Implementation	64
15.	REFERENCE LIST	65

LIST OF TABLES

Table 1:	List of Abbreviations	11
Table 2:	Objectives, Endpoints, and/or Estimands	17
Table 3:	Summary of Study	19
Table 4:	Summary of Protocol Amendments.....	20
Table 5:	Summary of Treatment	22
Table 6:	Summary of Statistical Analysis Plan.....	28
Table 7:	Summary of Changes to the Planned Analyses	29
Table 8:	Summary of <Post Hoc Analyses> <Analyses After Unblinding> <Analyses After Database Lock>	30
Table 9:	Analysis Populations	36
Table 10:	Summary of Efficacy, Pharmacokinetic, Pharmacodynamic, and/or Biomarker Results	43
Table 11:	Summary of Safety Results: Placebo-Controlled and Open-Label Extension Periods	51
Table 12:	Vendors That Participated in Study <insert study number>	64

LIST OF FIGURES

Figure 1:	CONSORT Flow Diagram for Disposition of Participants	33
Figure 2:	Disposition of Participants.....	34
Figure 3:	Disposition of Participants.....	35

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4. LIST OF ABBREVIATIONS AND DEFINITIONS OF TERMS

NOTE TO WRITERS AND DQ REVIEWERS: Be sure this section is numbered as Section 4, not as Section 1.

List initialisms and abbreviations that are unfamiliar or that are specific to this study. For example, do not include common abbreviations such as AE, FDA, US, WHO, GCP, IRB, and so on. If in doubt about a specific abbreviation, include in the table. Abbreviations that are expanded (defined) in this list should be in lowercase unless they are proper nouns or typically use capitalization. If two different terms use the same abbreviation, spell one in full in the text and use the abbreviation for the other. Abbreviations in the table should be listed alphabetically.

In general, do not spell out (define) abbreviations at first use in text, tables, or figures. Do not abbreviate terms that appear only once in a document. For detailed and up-to-date guidelines on the use of abbreviations, see the **Style Guide**.

Table 1: List of Abbreviations

Abbreviation	Definition
α-syn	alpha-synuclein
ADA	antidrug antibody
COVID-19	the disease caused by a new coronavirus called SARS-CoV-2
MMSE	Mini-Mental State Examination
SMT	Study Management Team

5. ETHICS

5.1. Independent Ethics Committee and/or Institutional Review Board

Provide information on site ethics committee (EC) approval. Confirm that the protocol and any of its amendments, as well as information provided to participants, were reviewed by an IEC and IRB.

Investigators obtained EC approval for the study protocol and all amendments. A list of ECs for sites participating in this study is included in [Appendix 16.1.3](#).

5.2. Ethical Conduct of the Study

Provide information on the study's compliance with the United States Code of Federal Regulations (CFR) and/or the International Council for Harmonisation (ICH) Tripartite Guidelines on Good Clinical Practice (GCP). If there were any Serious Breaches of GCP in the study, these should be mentioned here, with a cross-reference to a more detailed description in Section 10.7 (Protocol Deviations). If the study was also performed in China, add reference to being performed in accordance with GCP guidelines defined in NMPA 2020 No. 57.

This study was performed in accordance with Title 21, United States Code of Federal Regulations (CFR) Parts 50, 54, 56 and 312 Subpart D; the International Council for Harmonisation (ICH) Guideline on Good Clinical Practice (E6); the European Union Clinical Trial Directive 2001/20/EC or Clinical Trial Regulation 536/2014; and the ethical principles outlined in the Declaration of Helsinki; and/or, where applicable, the European Directive 2001/20 in relation to Good Clinical Practice (GCP) in the conduct of clinical trials on medicinal products for human use and Directive 2005/28 on Good Clinical Practice for investigational medicinal products for human use.

5.3. Participant Information and Consent

Provide information on how and when informed consent was obtained in relation to participant enrollment. Add any study-specific or program-specific information here as it relates to informed consent, such as for participants with visual impairment, disability due to stroke, or other specialized populations.

Written informed consent was obtained from each participant prior to evaluations performed for eligibility. Participants were given adequate time to review the information in the informed consent and were allowed to ask, and have answered, questions concerning all portions of the conduct of the study. Participants were provided with a copy of the signed and dated Informed Consent Form (ICF).

6. INVESTIGATORS AND STUDY ADMINISTRATIVE STRUCTURE

6.1. Investigators

State the number of sites that 1) were activated for study participation; 2) that screened participants; 3) that randomized participants, and 4) that dosed participants.

Provide information on the Principal Investigator/Coordinating Investigator of the study: name, credentials, and institution (affiliation, city, state, country). The Sponsor's Study Medical Director and Clinical Trial Lead (CTL) can provide this information. All CSRs for studies started in or after April 2002 must be signed by the Principal Investigator/Coordinating Investigator (Appendix 16.1.5). Provide a statement on training of sites prior to activation.

<X> sites were activated for participant screening, of which <X> screened participants, <X> randomized participants, and <X> dosed participants.

A list of all Investigators and their affiliations is presented in [Appendix 16.1.4](#).

Training was provided to study sites prior to screening participants (e.g., at study initiation visits and Investigator meeting).

6.2. Study Committees

Provide information on all study committees (e.g., Advisory Committee, Data Monitoring Committee, Safety Monitoring Committee). Include information on the committee's role, committee members, how often they met, if they were blinded, etc. Use subheading level 3 for each committee, as appropriate.

If applicable, any guidelines/charters associated with the study may be referenced and copies provided to Regulatory Operations for inclusion in Appendix 16.1.10. It may also be appropriate to present a list of study committee members in Appendix 16.1.4.

6.3. Vendors

Reference the vendor table that appears in Section 14.4 (Additional Information on Study Design and Implementation), as follows:

A list of vendors and subcontractors that had responsibilities for study conduct, and data handling, analysis, and reporting is presented in [Table 12](#).

6.4. Authors of the Clinical Study Report

Add and/or delete authors and/or functional areas as applicable to the study. Include only 1 author per functional area; this should be the person accountable for the CSR. It is not necessary to indicate if an author was a contractor; all contractors will be presented in the vendor list.

The following individuals were authors of the clinical study report:

Study Medical Director: <name and credentials>

Global Safety Officer: <name and credentials>
Biostatistician: <name and credentials>
Pharmacokineticist: <name and credentials>
Medical Writer: <name and credentials>

7. INTRODUCTION

This section should be 1 to 3 pages maximum in length. Trim excessively long text to provide only the relevant material. Provide a brief overview (1 paragraph for each topic) of the indication (e.g., pathology, epidemiology, clinical features, diagnostic criteria, available treatments) and a description of why the study is being conducted. Separate, unnumbered headers can be used for each topic if desired. Provide the Study Rationale information from the protocol here. Change the tense (e.g., future to present or future to past) of content copied from the protocol, as appropriate. Reference any external guidelines that were followed and/or a high-level summary if specific health authorities were consulted in designing the study (avoiding commercially confidential information [CCI], such as meeting minutes, that would be better reserved for CTD documents). Briefly summarize additional new information since the start of the study (e.g., safety updates, early termination) and an explanation of partial reporting (e.g., biomarkers to be provided later as addenda). Especially for studies with multiple consecutive analyses and reports, the introduction should contain a clear statement on which cutoff dates are used for the analyses reported (e.g., date of database release).

Do *not* present the following:

information related to mechanism of action unless Legal has indicated there are no issues with intellectual property

activity in indications not evaluated in the present study

yet-to-be labeled dosing regimens

Do *not* repeat description of the study from the study title in the introductory text. For example, begin as follows:

<The aim of this study was to demonstrate the efficacy of BIIBxxx in delaying progression of the clinical signs and symptoms of <i>indication. . . </i>.>

<The clinical study report for Study 123MS301 comprises the present report and final analysis results for the double-blind period and the interim analysis results of the open-label extension period. The interim analysis occurred as of the data cutoff date of <i>date>.>

Then, place background information about the investigational drug at the end of the section. For example:

<BIIBxxx is a humanized monoclonal antibody of subclass . . . >

8. STUDY OBJECTIVES, <ENDPOINTS> <AND/OR> <ESTIMANDS>

Copy all study objectives and endpoints (or estimands, if defined for the study) from the protocol; minor wording changes may be made for consistency or style compliance (such as terminology or abbreviations). Do not make any changes that might alter the meaning. Include tertiary/exploratory objectives and endpoints. If there were no secondary or additional/exploratory objectives, these subsections may be deleted. If an objective is not addressed in the CSR, it may be excluded from this section. However, explain its absence in either Section 9.2.1 (Changes to the Study Conduct) or Section 9.7.2 (Changes to the Planned Analyses).

The following is the suggested tabular format, including statistical methods, analysis populations, and links to relevant SAP sections. Title the table as appropriate (e.g., if there are separate treatment periods).

Table 2: Objectives, Endpoints, and/or Estimands

Primary Objectives	Primary Endpoints/ [Estimands]
<objective(s) as listed in protocol>	<endpoint(s)/[estimands] as listed in protocol>
Secondary Objectives	Secondary Endpoints/ [Estimands]
<objective(s) as listed in protocol>	<endpoint(s)/[estimands] as listed in protocol>
Tertiary/Exploratory Objectives	Tertiary/Exploratory Endpoints/ [Estimands]
<objective(s) as listed in protocol>	<endpoint(s)/[estimands] as listed in protocol>

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9. INVESTIGATIONAL PLAN

Include an introductory statement with the study number and study title and date of the protocol under which enrollment began (i.e., when the first participant was screened). Reference Appendix 16.1.1 for protocols and amendment summaries and Appendix 16.1.2 for sample CRFs. For example:

“Study < study number> was a <Phase 2> <multicenter, randomized, double-blind, placebo-controlled, or as applicable> . . . The first participant was screened on <dd-mmm-yyyy>. See Appendix 16.1.1 for protocols and amendment summaries. See Appendix 16.1.2 for sample CRFs.”

9.1. Overview of Study Design

Provide the study schema (if applicable) and description of overall design from the protocol in this section.

Present the main methodological features of the study, including the type of study; method of blinding; randomization; number of participants for enrollment; approximate number of sites and treatment groups; treatment course; sequence and duration of all treatment periods; expected duration of participation in the study; additional treatment, if applicable; special features or circumstances of the study; end of study; study stopping; and any planned future study or ongoing study that is available to extend a participant’s treatment. For clarity, this information may be presented in a summary table (see example table below).

Use a schematic to clarify the study design, if appropriate.

References to the relevant figure and the protocol appendix are appropriate. For instance:

The study design is shown in <Figure X.> Additional details are available in the protocol (Appendix 16.1.1, Protocol <study number>, Version X, Section X.X).

Example of study summary table:

Table 3: Summary of Study

Parameter	Description
Study Design	
Study Treatment (dose, regimen, route)	BIIBxxx
Control Drug (dose, regimen, route)	
Duration	
Diagnosis Inclusion Criteria	
Primary Objectives	Refer to Section 8
Primary Endpoints/[Estimands]	Refer to Section 8
Secondary Objectives	Refer to Section 8
Secondary Endpoints/[Estimands]	Refer to Section 8
...	

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9.2. Discussion of Study Design

Briefly discuss the rationale for the choice of study design (including any randomization) and choice of any control treatment. Discuss the scientific rationale, which may be taken from the protocol. This section should not restate the description of the study design, but rather should discuss why it was appropriate for the study. Present key advantages of the study design, as applicable. Discuss any agreements between the Sponsor and regulatory authorities that are relevant to the study.

References to the relevant portion(s) of the protocol are appropriate. For instance:

The scientific rationale for features of the study design, including chosen control group(s), dose(s), and endpoint(s), as applicable, are discussed in further detail in the Study Rationale section of the protocol (Appendix 16.1.1, Protocol <study number>, Version X, [Section X.X](#)).

9.2.1. Changes to the Study Conduct

Briefly address changes in the conduct of the study in this section (changes to the planned analyses are to be described in Section 9.7.2; detailed information about substantial changes is provided in the protocol amendments in Appendix 16.1.1). Changes in the conduct of the study can include, but are not limited to, dropping a treatment group, changes in entry criteria or drug (study treatment or non-study treatment) dosages, adjusting the sample size, and changes in assessment schedules. Provide information on the number of times the protocol was amended and the reasons for each amendment. If any changes were made to the study conduct without a corresponding protocol amendment, state why it was agreed not to amend the protocol and the role that approved this decision.

A table may be included to summarize the global and country-specific amendments. For example:

Table 4: Summary of Protocol Amendments

Protocol Version	Date	Primary Reason(s) for Change
Original (Version 1)		
Version 2		
Version <X>		
Version <X>		
...		

State that all protocol amendments were submitted and approved by the study sites' ethics committee. Reference Appendix 16.1.1.

If major changes to the protocol were made (e.g., changes to endpoints, schedule of events, design), it may be important to specify the number of participants that were screened under each protocol version. The decision to include that information should be made by the SMT. The information may be presented in a table.

Discuss with the SMT if letter notifications were sent to the sites. If these letters were used for minor (e.g., administrative) changes, these do not have to be discussed in the CSR.

If applicable, add a sentence if a continuation/extension study was not conducted as planned.

Select appropriate options as follows:

Option 1:

Changes to the conduct of the study that were implemented by protocol amendment(s) are described in (Appendix 16.1.1, Protocol <study number>, [Version X](#)). Additional changes in the conduct of the study are described below.

Option 2:

All changes to the conduct of the study were implemented by protocol amendment(s), as described in (Appendix 16.1.1, Protocol <study number>, [Version X](#)).

Option 3:

There were no changes to the conduct of the study.

9.2.2. General Trial Conduct Mitigation Strategies During Public Health Emergencies

Omit this section if not applicable to the study.

The impact of certain public health emergencies (e.g., the COVID-19 pandemic) on study completion was examined (https://health.ec.europa.eu/system/files/2022-02/guidanceclinicaltrials_covid19_en_1.pdf; <https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/covid-19-related-guidance-documents-industry-fda-staff-and-other-stakeholders>]. As relates to public health emergencies, see Section [9.7.2.1](#) for changes to the statistical analyses, Section [10.7](#) for a discussion of protocol deviations, and Section [12.2.2](#) for a discussion of adverse events.

9.3. Study Population

Provide a high-level description of the study population, including the most important inclusion criteria, with a cross-reference to the protocol in Appendix 16.1.1. Also provide a high-level description of the reasons participants were to be withdrawn from the study or were to have study treatment discontinued. State that the reasons were to be recorded in the participant's CRF. Cross-reference to the latest version of the study protocol (as applicable) in Appendix 16.1.1 for details about these topics.

For instance:

This study enrolled participants with <indication> who were between 18 and 65 years of age, inclusive; had an MMSE score of ≥ 20 at Screening; and whose concomitant medications had been stable for at least 30 days prior to Screening. A complete list of inclusion and exclusion criteria are provided in Appendix 16.1.1, Protocol <study number>, Version X, [Section X](#) and [Section Y](#).

The specific criteria and procedures for early discontinuation from study treatment or for withdrawal from the study are described in Appendix 16.1.1, Protocol <study number>, Version X, [Section X](#).

9.4. Treatments

Include an introductory statement briefly summarizing the number of treatment groups, the study treatment(s) and regimen each group was to receive, and the frequency of treatment.

In tabular format, present information on treatments administered, rationale for study dose, method of treatment assignment, selection and timing of doses, dosage modification, blinding and other measures to minimize bias, prior and concomitant therapy, and any other relevant information (e.g., comparators) from the protocol, as shown in the following example. The source of any placebo, active control, combination therapy, or comparator product should be provided. Any modification of these products from their usual commercial state should be noted, and the steps taken to ensure that their bioavailability was unaltered should be described. Any use of test materials past their expiry date should be noted, and the participants receiving them should be identified. Any concerns regarding shipment or storage conditions should also be noted.

Any additional information that also appears in the protocol should reference the protocol in Appendix 16.1.1.

Following the table, consider inserting any figures that depict dose-escalation or crossover schemes.

Table 5: Summary of Treatment

Parameter	Description	Protocol and Section
Treatment Administered	XXXX mg of BIIbxxx, once every X weeks (QXW), by XX administration. In addition to dosing regimen, briefly describe study treatment (e.g., formulation, strength, number of batches [from Biostats; do not list specific batch numbers]). If required, refer to Appendix 16.1.6.	Protocol <number> VX Section X
<Comparator or Placebo Administered>	<description>	Protocol <number> VX Section X
Rationale for Study Dose	Provide rationale for selection of study dose(s), including dose escalation. Data from previous studies found . . . was predicted to give a high likelihood of demonstrating efficacy in the treatment of xxx.	Protocol <number> VX Section X

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Parameter	Description	Protocol and Section
Method of Treatment Assignment	All participants were centrally randomized using . . . If randomization code was developed by a vendor, refer to the vendor list in Section 14.1 (do not name the vendor here).	Protocol <number> VX Section X
Dosage Modification	Describe instances where dosage or administration of study treatment was to be modified or temporary withheld and whether assessment schedule was to be modified as a result. Dosage modifications were not permitted.	Protocol <number> VX Section X
Blinding	Describe blinding procedures, including precautions taken to ensure adequate blinding was maintained throughout the study. Describe who was blinded and unblinded at the study site. Describe blinding of study treatment (e.g., appearance, preparation, etc.). If blinding and unblinding methods are described in an appendix, ensure the text aligns with the appendix. Alignment with the SAP and Unblinding Plan should also be taken into consideration. All study personnel remained blinded . . .	Protocol <number> VX Section X
Prior and Concomitant Therapy	List therapies (medications, procedures) allowed or prohibited prior to study entry and during the study. Give details such as drug name, dosage, washout periods, and reasons for allowing or prohibiting. Describe washout periods. Describe where information was captured. Medications taken within 4 weeks prior to study treatment . . .	Protocol <number> V7 Section X

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9.5. Study Activities and Assessments: <Efficacy, Pharmacokinetics, Pharmacodynamics, Biomarker, Safety, Immunogenicity>

Do not insert study activities charts (i.e., from the protocol) in this section. Do not include the visit schedule or describe specific assessments or tests in this section. Instead, refer to the applicable sections in the protocol in Appendix 16.1.1. For example:

The <efficacy, PK, PD, safety, immunogenicity, or other variables> assessments used in the study to evaluate <treatment name or compound number>, including their schedule and measurement/collection methods, can be found in the Schedule of Activities of the protocol. A description of each assessment is presented in Section X (efficacy), Section Y (PK), and Section Z (safety) of the protocol (Appendix 16.1.1, Protocol <study number> Version X, [Section X](#), [Section Y](#), [Section Z](#)). Definitions, collection, reporting, and assessment of safety information during the study are detailed in the protocol (Appendix 16.1.1, Protocol <study number> Version X, [Section X](#)).

9.6. Data Quality Assurance

The quality assurance and quality control systems used to ensure the quality of the data should be described in brief.

Documentation of interlaboratory standardization methods and quality assurance procedures, if used, should be provided in Appendix 16.1.10. Any steps taken at the study site or centrally to ensure the use of standard terminology and the collection of accurate, consistent, complete, and reliable data (such as training sessions, monitoring visits, instruction manuals, data verification, SAE reconciliation, data cleaning, database locking, use of central laboratories, centralized ECG reading, or data audits) should be described.

If an independent internal or external auditing procedure was used, it should be mentioned here and described in Appendix 16.1.8. Include statements of error rate and other similar details or any audit procedure description, if applicable.

9.6.1. Quality Risk Assessment

A quality risk is defined as any aspect of a clinical trial with the potential to have an adverse impact on participant safety, rights, or well-being, or on the data integrity or scientific value of the trial.

For all clinical studies, state whether a quality risk assessment was performed prior to protocol approval and/or first patient in (FPI), and that study-specific key quality indicators (KQIs) and quality tolerance limits (QTLs) were set based on the quality risk assessment (see DEV-SOP-2654). Summarize which KQIs and QTLs were established prior to the start of the study and provide a cross-reference to Appendix 16.1.10 for output/data from KQI monitoring. The following wording can be used:

Prior to protocol approval, a study-specific Quality Risk Assessment was performed, and KQIs and QTLs were set for the study based on that assessment. For a copy of the Quality Risk Assessment, refer to the Trial Master File. For results of study-specific KQI monitoring, see

Appendix 16.1.10. <If applicable:> For a brief discussion of the KQI monitoring results, QTLs exceeded, and remedial actions taken, see [Section X](#).>

It should be noted whether Investigator meetings or other steps were taken to prepare Investigators on how to maintain and/or proactively assess their compliance in line with the protocol and KQI/QTLs, as well as to standardize intersite procedures.

If a quality risk assessment was not conducted for this study, supply a statement to this effect along with a specific rationale for why it was not performed. **Do not delete this section if a quality risk assessment was not performed.**

9.6.2. Key Quality Indicator Monitoring

For all clinical trials (see DEV-SOP-2654), provide a brief discussion of the results from study-specific KQI monitoring. Of the two QTLs – amber and red – report any crossing of red QTLs. Consult your SMT to determine whether any study-specific red QTLs have been crossed. (Red = QTL at which a state of control is lost, resulting in likely impact on participant safety, rights, and well-being or on data integrity/scientific value of the study Amber = QTL at which predetermined actions must be taken to regain a state of control so as to prevent a potential impact on participant safety, rights, and well-being or on data integrity/scientific value of the study.) In cases where the red QTL has been crossed but this is not reported in the CSR (e.g., the QTL was subsequently changed/removed or found not to be meaningful, or other reasons), provide a justification in the TMF (e.g., in SMT minutes, CSR development team meeting minutes, Note to File). Describe important deviations from the predefined QTLs (defined as a KQI metric that indicates a likely impact on participant safety, rights, or well-being, or on the data integrity or scientific value of the study), and remedial actions taken to bring the study back into compliance. If there were no important deviations, the following statement can be used:

<The study defined and monitored the following key quality indicators: <list indicators>. There were no important deviations from predefined QTLs.>

If key quality indicator monitoring was not conducted for this study, supply a statement to this effect along with a specific rationale for why it was not performed. For programs that do not typically conduct key quality indicator monitoring for individual studies, the MW Lead should discuss this ICH requirement with the CTL. **Do not delete this section if a quality risk assessment was not performed.**

9.6.3. Data Collection

The information in brackets should be supplied by Clinical Operations. Revise as needed if remote monitoring was performed.

Prior to the first participant being screened at each site, initiation visits were held for the purpose of training site staff on the data collection tool, procedures, tests and evaluations to be used, conducted, or assessed in the study. Instruction manuals were provided to the sites. <Describe Investigator Meetings here if applicable.>

During the study, Clinical Monitors visited each site at regular intervals. All monitoring visits were conducted according to the applicable ICH and Good Clinical Practice (GCP) guidelines to ensure protocol adherence, quality of data, drug accountability, compliance with regulatory requirements, and continued adequacy of the study site and its facilities. During these visits, data

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were source verified according to Standard Operating Procedures. If the Information on the CRF was not consistent with the source data, or if it was questionable in any way, the Clinical Monitor requested a clarification or correction to the item by site personnel.

9.6.4. Data Management and Quality Control

For Functional Service Provider (FSP) or Full-Service EDC studies use the following text. The information in brackets should be provided by Clinical Data Management. If data management and/or imaging was performed by a vendor, provide a reference to the vendor list in Table X. The vendor should not be named here.

Data were managed using <Medidata-Rave® version X, Oracle Clinical/Remote Data Capture (RDC®) version 4.6.2 update version as required> <or Oracle Clinical-Inform® version 4.6.2 update version as required> <or >>, an EDC collection tool. This software application is compliant with 21 CFR Part 11, Electronic Records; Electronic Signatures. The software was formally validated according to internal policies. All users were trained on the software application, and none were granted access until they were certified as trained. Investigational site personnel entered participant clinical data directly into EDC. If the data did not meet predetermined parameters, a discrepancy was displayed in the system and corrections to the data were made by investigational site personnel. Discrepancy responses were reviewed by data management and closed, where applicable. QC data reviews for logic and consistency were performed prior to database lock. CRFs were electronically source document verified by the CRA and approved by the Investigator in EDC. After database lock, the Investigator received all participant data and associated discrepancies for archiving at the investigational site.

Choose one, as applicable:

<Clinical laboratory data were entered into the EDC by investigational site personnel.>

OR

<These data were electronically loaded into the EDC by the clinical programmer. Standard quality control checks were performed.>

For Functional Service Provider (FSP) or Full-Service Paper studies use the following text. The information in brackets should be provided by Clinical Data Management.

Data management was performed using <Oracle Clinical version 4.5.3 update software and version as required>, a data capture collection tool. This software application is compliant with 21 CFR Part 11, Electronic Records; Electronic Signatures. The software application was formally validated according to internal policies. All users were trained on the software application, and none were granted access until they were certified as trained. CRFs were completed for each participant and submitted for database entry. If the data did not meet predetermined parameters, the system displayed a discrepancy, and requested corrections (Data Clarification Forms) were sent to site personnel for completion. QC reviewed data for logic and consistency prior to database lock. CRFs were source document verified by the CRA and approved by the Investigator.

Choose one, as applicable:

<Clinical laboratory data were entered onto a CRF and submitted for entry into the data capture tool.>

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OR

<These data were electronically loaded into data capture collection tool. Standard quality control checks were performed.>

9.6.5. Audits

In the following paragraph, state the number of sites audited as provided by R&D Quality Compliance Audit:

As part of the Sponsor's quality assurance program, <number of sites> study sites were audited for compliance with GCP. For a list of sites audited during this study, see [Appendix 16.1.8](#).

9.7. Statistical Methods Planned in the Protocol

9.7.1. Statistical Analysis Plan

Provide a summary of the SAP, preferably in tabular format (*see example table below, and include parameters applicable to the study*); refer the reader to the final SAP or to the protocol for detailed information regarding planned analyses.

The SAP is summarized in [Table 6](#).

<Changes to the planned analyses are described in Section [9.7.2](#).> For a detailed description of the planned analyses and determination of sample size, refer to the final version of the SAP (Appendix 16.1.9, Statistical Analysis Plan, [Version X](#)).

Table 6: Summary of Statistical Analysis Plan

Primary Endpoints/ [Estimands]	Statistical Method	Analysis Population	SAP Section (Appendix 16.1.9)
<endpoint(s)/[estimands] as listed in protocol>		<population(s) as listed in protocol>	<link to relevant SAP sections>
Secondary Endpoints/ [Estimands]	Main Statistical Method	Analysis Population	SAP Section
<endpoint(s)/[estimands] as listed in protocol>	<method(s) as listed in protocol>	<population(s) as listed in protocol>	<link to relevant SAP sections>
Tertiary/Exploratory Endpoints/ [Estimands]	Main Statistical Method	Analysis Population	SAP Section
<endpoint(s)/[estimands] as listed in protocol>	<method(s) as listed in protocol>	<population(s) as listed in protocol>	<link to relevant SAP sections>

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9.7.2. Changes to the Planned Analyses <Prior to Unblinding or Database Lock>

Use a summary table to list/describe changes to the planned analyses (*see example table below, and include parameters applicable to the study*). Characterize the timing of analysis changes relative to unblinding and availability of outcome results. Describe any changes in analyses that occurred after completion of the final SAP but before unblinding of the data. Changes made after study unblinding should be described separately in Section 9.7.3, Changes Following Study Unblinding and Post Hoc Analyses.

Table 7: Summary of Changes to the Planned Analyses

Planned Analysis	Change to Planned Analysis
...	

Select the appropriate text option:

Option 1:

Changes in the planned analyses for the study that were implemented by SAP [or protocol] amendment(s) are described in Appendix 16.1.9, Statistical Methods [and/or Appendix 16.1.1, Protocol <study number>, [Version X](#), if applicable]. Changes made after the final SAP and before unblinding or database lock (for open-label studies) are described here.

Option 2:

All changes in the planned analyses for the study were implemented by SAP [or protocol] amendment(s), as described in Appendix 16.1.9, Statistical Methods [and/or Appendix 16.1.1, Protocol <study number>, [Version X](#), if applicable].

Option 3:

There were no changes in the planned analyses for the study.

9.7.2.1. Public Health Emergency-Related Changes to the Statistical Analyses

If the SAP was modified due to a public health emergency, provide details here using a table to summarize any modifications to the statistical methodology.

Or choose one of the following wording options, as applicable:

Option 1:

<Public health emergencies did not substantially impact the data and/or scientific integrity of the study, so no additional or unplanned analyses were required to demonstrate the study objectives.>

Add cross-references to additional in-text tables or to appendix tables and listings to describe the effects of public health emergencies on study conduct and/or analyses.

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Option 2:

<A discussion of AEs and other disposition and deviation data related to public health emergencies is included in Section 11.>

Cross-reference any accompanying output, such as in-text tables or tables and listings in appendices.

9.7.3. <Changes Following Study Unblinding/Database Lock and Post Hoc Analyses>

Delete this section if not applicable.

Use a summary table to briefly list/describe changes to the planned analyses after database lock and post hoc analyses (see example table below and include parameters applicable to the study).

Table 8: Summary of <Post Hoc Analyses> <Analyses After Unblinding> <Analyses After Database Lock>

Analysis	Analysis Method	CSR Location of Results
...		

10. STUDY PARTICIPANTS

GENERAL NOTES FOR ALL RESULTS SECTIONS (These instructions are repeated at the beginning of Sections 10 and 11 in case different writers are assigned to different sections of the CSR.)

Present the results from the statistical outputs with objective and balanced presentation.

In general, text should provide high-level summary statements about and interpretations of key points. Text should not recapitulate the data that are presented in tables. Rather, text should comment on what is important to notice in the table(s).

Tables directly associated with or in support of information presented in the text should be cited in the main text. Other generated outputs (e.g., tables or listings containing supplementary relevant data or analyses specified in the SAP but not specifically discussed in text) do not need to be cited in the main text but should be set off from the text, e.g., in a box. For example:

Additional relevant data: <list tables from Section 14; list table numbers>

Relevant participant listings: <list participant listings from applicable appendices, e.g., Appendix 16.1.7, [Table 1](#); Appendix 16.2.1, [Table 1](#); and so on>

NOTE: Not all generated outputs included in the CSR appendices need to be cited in the text (e.g., analyses specified in the SAP but not specifically discussed in text).

If using a hand-tabulated table in text cannot be avoided for description of the results of pre-specified, ad hoc, or post-hoc analyses, be careful not to omit any information that would change the interpretation of the results. Hand-tabulated tables must be reviewed and approved by the appropriate functional representative. NOTE: Hand-tabulated tables for summaries of results are discouraged and should be avoided in CSRs. Discuss this point, including alternatives, with the SMT and Biostats.

Minimize use of participant-identifiable information in the text. Narratives and participant data listings should be presented in Appendix 16.2; cite the applicable appendix in the text.

10.1. Disposition of Participants

Provide a clear accounting (by treatment group, if appropriate), of all participants who were screened (if available) and randomized/treated in the study. Clarify by using figures and/or tables in the text (see examples in Figures 1 through 3, below). Select the figure which best fits the study design and populations tested. If the SMT decides that this study report will be used to develop a manuscript for publication, the CONSORT diagram ([Figure 1](#)) is preferred. These diagrams may be modified further in Visio to accommodate changes (fewer/additional groups, addition of screen failures, additions to the list of reasons for discontinuation from study, etc.).

This section can be further subdivided into subsections that discuss accountability in study-specific subpopulations (e.g., Participant Accountability: Placebo-Controlled Period, Participant Accountability: Open-Label Extension Period). Add the number of participants discontinued due to public health emergency-related events, as applicable.

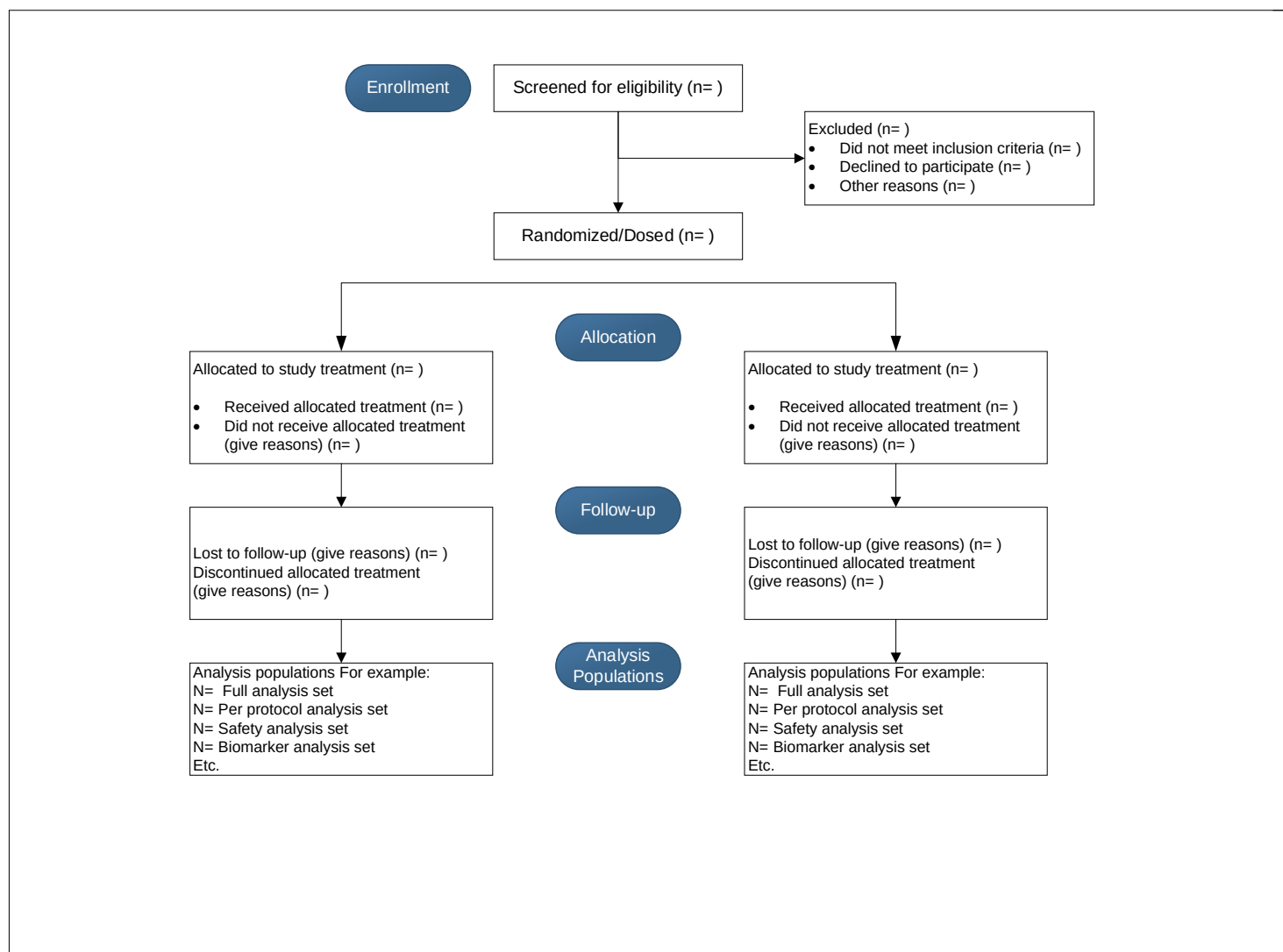
State the date the first participant was treated, the end of study date or (for ongoing studies) date of data cutoff, and the date of early study termination (if applicable). These dates must match the corresponding dates on the title page and in the Synopsis section.

If possible (i.e., 5 or fewer sites), provide the number of sites (per country for multi-country studies), and the number of participants screened at each site. For more than 5 sites, refer to Appendix X.

Be sure to include a listing in Appendix 16.2 of all participants (with participant IDs) who withdrew from the study after enrollment, broken down by site and treatment group, and providing the specific reason for discontinuation, and the duration of treatment before withdrawal.

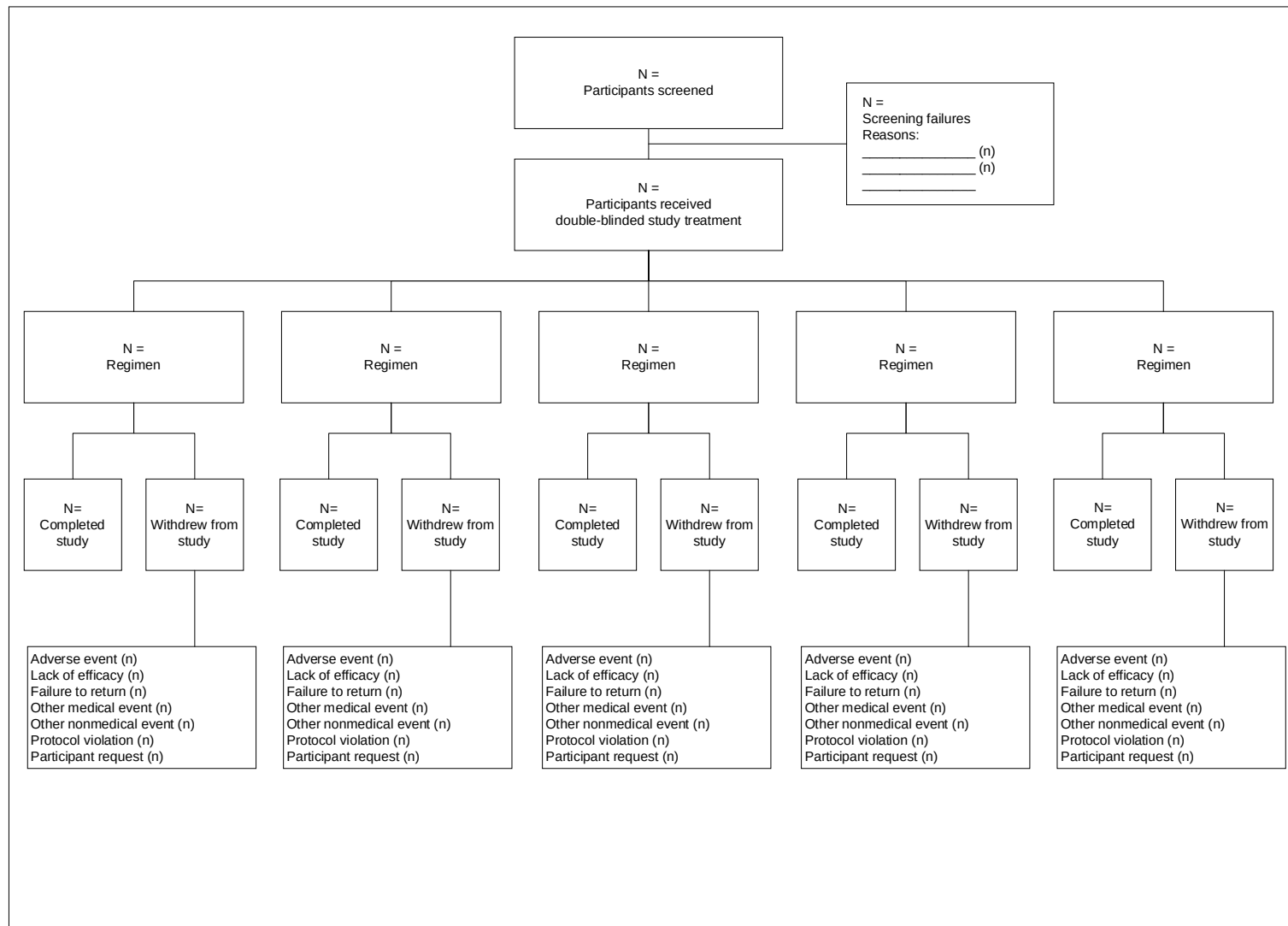
Participant accountability should also be described in the corresponding Results portion of the Synopsis section; be sure the information between the two sections matches.

Figure 1: CONSORT Flow Diagram for Disposition of Participants



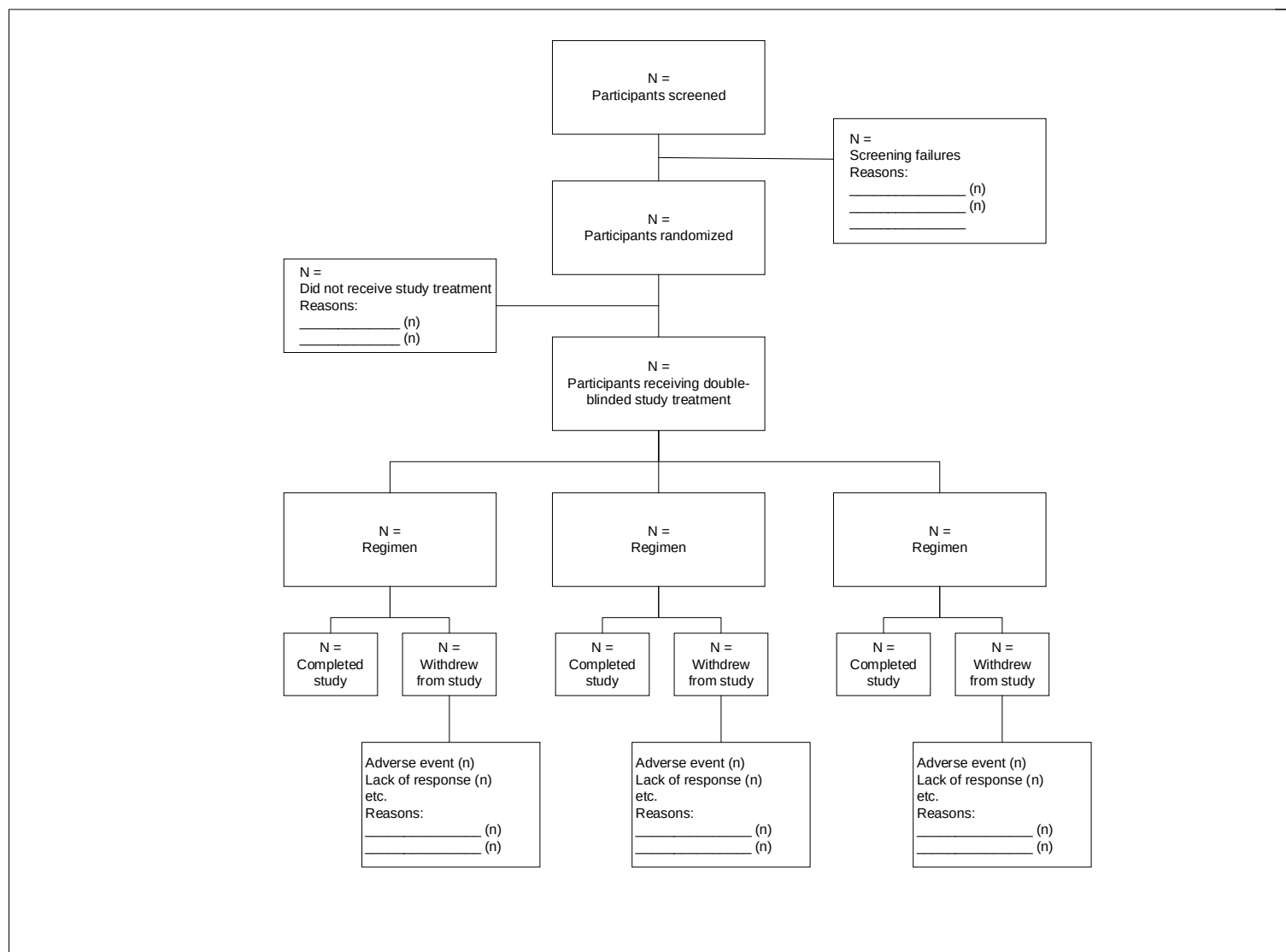
Source: <XXX>
 N = number of participants

Figure 2: Disposition of Participants



Source: <XXX>
 N = number of participants

Figure 3: Disposition of Participants



Source: <XXX>

N = number of participants

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10.2. Study Populations Analyzed

Define precisely which participant populations were included in each analysis. It should be made clear, if not defined in the study protocol, when (relative to study unblinding) and how criteria for the data sets analyzed were developed. Generally, even if the proposed primary analysis is based on a reduced subset of the participants with data, there should also be an additional analysis using all randomized (or otherwise entered) participants with any on-treatment data. Include a reference to the SAP for detailed information.

Discuss study-specific subgroups that were to be analyzed (e.g., analyses by age group, sex, weight, prior history of disease, etc.) as applicable. Add categorical analysis (demographics, etc.) for each subgroup as needed.

Present data from the analyses performed on subgroups (e.g., disposition of participants, prior therapy, exposure to study treatment, concomitant medication, protocol deviations).

Be sure to include a tabular listing of all participants, visits, and observations excluded from the efficacy analysis in Appendix 16.2.3 and reference this appendix for details.

The reasons why certain participants were excluded from a given analysis population (e.g., the Per-Protocol Population) should be summarized.

Table 9: Analysis Populations

Population	Definition
<ITT population>	
<Per-Protocol>	
<Biomarker Population>	
<Safety Population>	

10.3. Demographics

Present demographic characteristics (e.g., age, sex, race, height, weight, body surface area) and discuss their balance in relation to treatment group, if applicable. Include any other factors, such as stratification factors, that arose during the study and that might affect response. Demographics should also be presented in the corresponding Results portion of Section 2 (Synopsis). If applicable (e.g., in the case of large, controlled trials), consider stating that, in general, demographics were similar across treatment groups; then limit the discussion to any imbalances that were observed.

For studies with sites in the EU (for compliance with EU Clinical Trial Regulation), include the population of participants (including actual number of participants included in the clinical trial in the Member State concerned, in the EU, and in non-EU countries); age group breakdown, gender breakdown, as applicable. Omit for non-EU studies or if no direct synopsis posting.

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For clarity, subsections can be added for specific topics.

10.4. Baseline Disease Characteristics

Present data for relevant baseline disease characteristics. Discuss the balance in relation to treatment groups. Include any other factors, such as stratification factors, that arose during the study and that might affect response. This section should be deleted for studies in healthy volunteers. Baseline disease characteristics should also be presented in the corresponding Results portion of the Synopsis section of the CSR, as applicable.

If applicable (e.g., in the case of large, controlled trials), consider stating that, in general, baseline disease characteristics were similar across treatment groups; then limit the discussion to any imbalances that were observed.

10.5. Baseline Medical History

Present data for medical history and/or concurrent illnesses if this is useful. Include any other factors, such as stratification factors, that arose during the study and that might affect response. Baseline medical history should also be presented in the corresponding Results portion of the Synopsis section of the CSR, as applicable.

10.6. Prior and Concomitant Treatments

Before presenting results, cross-reference [Table 5](#) in Section 9 for information about dose modifications allowed per protocol, as applicable.

Present data for any relevant treatments (medicines, procedures) used prior to or during the study and discuss their balance in relation to treatment groups.

10.6.1. Prior <Current Indication> Treatments

Present data for relevant treatments (medicines, procedures) used for the current indication prior to the study and discuss their balance in relation to treatment groups.

10.6.2. Concomitant <Current Indication> Treatments

Present data for relevant treatments (medicines, procedures) during the study and discuss their balance in relation to treatment groups.

10.7. Protocol Deviations

Briefly summarize all the major protocol deviations (see DEV-SOP-811) per the classification described in the SAP (include cross-reference to Appendix 16.1.9, Statistical Analysis Plan, [Section X](#), as applicable). The summary should state that the list of deviations was generated after the protocol was finalized and include a description of the impact of the deviations on the study (at the participant level, study site level, or overall). Focus on the participant population as a whole and not on individual participants; instead, as appropriate, reference the appropriate appendix (Appendix 16.2.2) for a listing of protocol deviations by participant. (If it is necessary to refer to individual participants, can include participant IDs and age in the text but no other identifying information, such as country, date of birth, and so on.)

State whether the important deviations resulted in exclusion of any participant data from any analyses. State whether any important deviations resulted from serious noncompliance (see DEV-SOP-779) or other GCP-related issues.

If applicable, add text on the number of out-of-window visits or missed visits due to public health emergencies (e.g., the COVID-19 pandemic).

If predefined quality tolerance limits (QTLs) were included in the RMP, and if an important deviation from a QTL also qualified as an important protocol deviation, describe it here.

The following is sample text, which may be modified to fit the study.

Deviations were departures from the current approved version of the protocol.

A list of potential protocol deviations was generated by the SMT after the protocol was finalized. Major deviations were considered changes from the protocol that could affect data integrity or participant safety. Major deviations included < insert study-specific examples here>. Minor deviations included <insert study-specific examples here>. Prospective (planned) protocol deviations were not permitted.

All deviations (major and minor) were captured by the Clinical Monitors on a Protocol Deviation log. Periodically during the course of the study, key study team members (e.g., Biostatistician, Statistical Programmer, Data Manager, Operations Lead, and Medical Director) reviewed protocol deviations identified from the clinical database via programmed data listings and edit checks in addition to deviations captured in the Protocol Deviation log to identify trends that required retraining at sites or a protocol amendment, and to identify major protocol deviations and determine if they affected the planned statistical analysis.

A summary of the major deviations that occurred on study is presented in [Table X](#). The most common major deviations were <list deviations>. A listing of protocol deviations identified within the clinical database and the Protocol Deviation log are provided in [Appendix 16.2.2](#).

10.7.1. Serious Breaches of Good Clinical Practice

Serious breaches of GCP should be described, as well as their effect on the data (e.g., exclusion of data from sites with GCP violations). A serious breach is one that is likely to affect to a significant degree (a) the safety, physical, or mental integrity of trial participants, or (b) the

scientific value of the trial. (See DEV-SOP-1899.) In the event that any audited site had GCP noncompliance issues, include a reference to Appendix 16.1.8 (Audited Sites).

10.7.2. Unblinding Events

Use any of the following subsections related to unblinding that apply to this study. Where applicable, provide cross-references to the Unblinding Plan in Appendix 16.1.9. If the unblinding event was related to a protocol deviation and/or a serious breach of GCP, provide a cross-reference to the applicable section(s) in the CSR. Provide a statement on the impact of unblinding events on the study, on data integrity, and/or on the analyses; cross-reference Section 11 to describe how/whether these participants were excluded from the analyses. List how many participants were unblinded by the Investigator, and (if a placebo-controlled or comparator study) state how many were randomized to which study treatment. State the reasons why Investigators chose to unblind the participants; provide details in Section 10.7.2.1 and Section 10.7.2.2.

If no unblinding events (whether planned or inadvertent) have occurred, provide a statement to that effect (e.g., “No events of planned or accidental unblinding occurred during this study.”) and omit the applicable level 4 heading.

10.7.2.1. In the Case of a Medical Emergency

Describe provisions for emergency decoding available to the Investigator and/or designated study site personnel (e.g., via IVRS). Provide an overview of the actions the Investigator was to take if knowledge of the participant’s treatment assignment might influence their clinical care, including prior to unblinding the treatment assignment (e.g., discuss the emergency with the Medical Monitor or other role, as appropriate). Describe how and where the Investigator documented the reasons for emergency medical unblinding, as well as any recommendations regarding divulging of the participant’s treatment assignment.

List how many participants were unblinded for a medical emergency, along with any other relevant details.

Provide a cross-reference to the Unblinding Plan in Appendix 16.1.9, along with a cross-reference to any other pertinent documentation.

10.7.2.2. Accidental Unblinding

Describe what actions were taken if a participant’s treatment assignment was accidentally unblinded, including who recorded the unblinding. Describe what actions were taken to prevent future occurrences of accidental unblinding, as applicable. Describe what actions were required to be taken if other relevant site personnel were unblinded along with the participant.

List any instances of accidental unblinding that occurred during the study (can be provided as a bulleted list).

Provide a cross-reference to the Unblinding Plan in Appendix 16.1.9, along with a cross-reference to any other pertinent documentation.

10.8. Exposure and Study Treatment Compliance

10.8.1. Exposure

Present data on the number of doses of study treatment received by participants per treatment group, time on treatment, and discuss any errors in the administration of study treatment and any withholding of doses, if applicable. Reference Section 10.7 for more details on protocol deviations, if applicable. Omit any subsections that are not applicable.

10.8.2. <Dose Modification>

Before presenting results, cross-reference the Summary of Treatment table (Section 9) for information about dose modifications allowed per protocol.

Summarize impact of dose modifications (dose finding/escalations, interruptions, reductions/increases, delays) to determine the degree to which safety/benefit can be assessed from the study.

10.8.3. <Compliance>

Before presenting results, cross-reference the applicable table in Section 9 for information about dose modifications allowed per protocol.

Describe and present data on the degree to which participants followed their intended study treatment regimen (may be expressed as a percentage).

11. EFFICACY <, PHARMACOKINETIC, PHARMACODYNAMIC AND/OR BIOMARKER> RESULTS

GENERAL NOTES FOR ALL RESULTS SECTIONS (These instructions are repeated at the beginning of Sections 10 and 11 in case different writers are assigned to different sections of the CSR.)

Present the results from the statistical outputs with objective and balanced presentation.

In general, text should provide high-level summary statements about and interpretations of key points. Text should not recapitulate the data that are presented in tables. Rather, text should comment on what is important to notice in the table(s).

Tables directly associated with or in support of information presented in the text should be cited in the main text. Other generated outputs (e.g., tables and listings containing supplementary relevant data or analyses specified in the SAP but not specifically discussed in text) do not need to be cited in the main text but should be set off from the text, e.g., in a box. For example:

Additional relevant data: <list tables from Section 14, e.g., [Table 46](#)>

Relevant participant listings: <list participant listings from applicable appendices, e.g., Appendix 16.2.1, [Table 1](#); Appendix 16.2.4, [Table 8](#)>

If using a hand-tabulated table in text cannot be avoided, be careful not to omit any information that would change the interpretation of the results. Hand-tabulated tables *must* be reviewed and approved by the appropriate functional representative. **NOTE: Hand-tabulated tables are discouraged and should be avoided in CSRs.** Discuss this point, including alternatives, with the SMT and Biostats.

NOTE: Not all generated outputs included in CSR appendices need to be cited in the text (e.g., analyses specified in the SAP but not specifically discussed in text).

Minimize use of participant-identifiable information in the text. Narratives and participant data listings should be presented in Appendix 16.2; cite the applicable appendix in the text.

Provide any results (not methods) from any statistical issues/findings/considerations (e.g., changes to the analyses, baseline imbalance, missing data, multiple comparisons). Subheadings may be inserted as applicable for relevant topics (e.g., Post Hoc Analyses; Adjustments for Covariates; Handling of Withdrawals, Discontinuations, or Missing Data; Interim Analysis and Data Monitoring; Multicenter Studies; Use of an “Efficacy Subset” of Participants; Examination of Subgroups; Regional Issues; and so on).

If not applicable to the study, state that these data were not collected and/or analyses were not performed on these endpoints.

Include or omit any of the following subsections as appropriate, depending on the analyses conducted. If estimands were implemented in the study, describe for each the applicable analyses (e.g., primary analysis, sensitivity analyses, supplementary analyses). Add subsections for any other applicable efficacy analyses.

Consider beginning the section with a tabular presentation that states primary and secondary objectives/endpoints and indicates the results (*see example table below*). Make the table as

detailed and informative as possible (e.g., mean change, range, determination of significance, etc.). If any p-value less than 0.05 is not considered significant (due to a closed testing procedure or other adjustment for multiplicity), indicate this with an asterisk (*) and footnote in the table below. Include a link to the locations in the CSR that present the results.

NOTE: See GENERAL TEXT INSTRUCTIONS FOR AUTHORS at the start of this template for guidance on manual creation of in-text tables.

Omit any subsections (heading level 2 and below) that are not applicable to the study.

Table 10: Summary of Efficacy, Pharmacokinetic, Pharmacodynamic, and/or Biomarker Results

Primary Objective(s)	Primary Endpoint(s)	Results	Sources

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Treatment groups should be compared for all critical measures of efficacy (primary and secondary endpoints; any pharmacodynamic endpoints studied). In general, the results of all analyses described in the protocol and an analysis including all participants with on-study data should be performed in studies intended to establish efficacy. The analysis should show the size (point estimate) of the difference between the treatments, the associated confidence interval, and, where used, the results of hypothesis testing.

Analyses based on continuous variables (e.g., mean blood pressure) and categorical responses (e.g., response rate) can be equally valid; both should be presented if both were planned and are available. If categories are newly created (i.e., not in the statistical analysis plan), the basis for them should be explained. Even if 1 variable receives primary attention (e.g., in a blood pressure study, supine blood pressure at Week X), other reasonable measures (e.g., standing blood pressure and blood pressures at other timepoints) should be assessed, at least briefly. In addition, the time course of response should be described, if possible. For a multicenter study, where appropriate, data display and analysis of individual sites should be included for critical variables to give a clear picture of the results at each site, especially the larger sites.

If any critical measurements or assessments of efficacy outcomes were made by more than one party, overall differences between the ratings should be shown. The assessments used should be clear in all analyses.

If applicable, state whether a planned analysis is to be conducted at another time, such that results are not included in this CSR (e.g., a follow-up for survival after 1 year).

State/describe the main findings of the efficacy evaluation *before* citing tables containing supporting data. Do not describe the measures here; these belong in the applicable section (add specific section number) of the protocol.

For the subsections below, repeat section headings as needed to include all primary and secondary efficacy endpoints.

11.1. Primary Efficacy Endpoint: <name of endpoint measure>

If deemed useful, consider beginning this text with an organizing statement in italic, such as the following; then continue with more specific points in subsequent paragraphs. Indicate in the organizing statement whether the measure/endpoint was met or unmet.

The primary efficacy endpoint of this study was met. A statistically significant reduction in . . . was observed, indicating

Prespecified sensitivity and supplementary analyses related to the primary endpoint should be included in this section after the primary efficacy analysis presentation and with subheadings, as appropriate.

Lastly, refer the reader to any post hoc analysis presented in Section 11.8 that relates to the primary endpoint analysis.

11.2. Secondary Efficacy Endpoint: <name of endpoint measure>

Characterize secondary efficacy endpoints. Use level 3 headings for multiple secondary endpoints. If deemed useful, consider beginning this text with an organizing statement in italic; then continue with more specific points in subsequent paragraphs.

For instance:

A statistically significant difference in motor function on the adjusted mean change from baseline . . . was observed. As a result, this key secondary endpoint was considered to be met.

Prespecified sensitivity and supplementary analyses related to the secondary endpoints should be included under the given level 3 section with level 4 subheadings, as appropriate.

Lastly, refer the reader to any post-hoc analysis presented in Section 11.8 that relates to these secondary endpoint analyses, as appropriate.

11.3. <Exploratory Efficacy Endpoints>

Present these endpoint findings separately under level 3 headings, as applicable.

11.4. <Pharmacokinetics>

If the study did not evaluate pharmacokinetics, state “Not applicable.”

Minimize use of participant-identifiable information in the text. Narratives and participant data listings should be presented in Appendix 16.2.

For CSRs presenting pharmacokinetic results, the Pharmacokinetics Report should be included in Appendix 16.1.10 and referenced in the text. If there are pharmacokinetic results, but no Pharmacokinetics Report, this should be explained in the text.

11.4.1. Study Treatment Dose, Concentration, and Relationship to Response

This section is optional; delete if not applicable to the study.

When the dose in each participant can vary, the actual doses received by participants should be shown and individual participant’s doses should be tabulated and included in Appendix 16.2. Although studies not designed as dose-response studies may have limited ability to contribute dose-response information, the available data should be examined for whatever information they can yield. In examining the dose response, the pharmacokineticist may calculate dose as body weight (mg/kg) or body surface (mg/m²).

PK parameters should be calculated and tabulated (Appendix 16.2.5) and, if possible, related to response.

Further guidance on the design and analysis of studies exploring dose-response or concentration response can be found in the ICH Guideline entitled “Dose-Response Information to Support Drug Registration.”

11.4.2. Drug-Drug and Drug-Disease Interactions

This section is optional; delete if not applicable to the study.

Any apparent relationship between response and concomitant therapy and between response and past and/or concurrent illness should be described.

11.5. <Pharmacodynamics>

If the study did not evaluate pharmacodynamics, state “Not applicable.”

If this section is applicable to the study, create level 3 headings to separately present and discuss PD findings, when appropriate.

11.6. <Biomarkers>

If the study did not evaluate biomarkers, state “Not applicable.”

If this section is applicable to the study, create level 3 headings to separately present and discuss biomarker findings, when appropriate.

11.7. Pharmacokinetic/Pharmacodynamic Analyses

If the study did not include PK/PD analyses, state “Not applicable.”

If this section is applicable to the study, create level 3 headings to separately present and discuss biomarker findings, when appropriate.

11.8. Post Hoc Analyses

Include all nonprespecified post hoc analyses performed for efficacy here.

Refer back to the relevant primary, secondary, and exploratory endpoint sections, as appropriate.

11.9. Statistical and Analytical Considerations in This Study

State in high-level terms what statistical or analytical issues arose during the study, whether adjustments were made for covariates (e.g., demographic or baseline measurements, concomitant therapy, or any other covariates or prognostic factors); dropouts or missing data; whether interim

analyses were performed; any extreme or opposite results among sites (for multicenter studies) including such possibilities as differences in study conduct, participant characteristics, or clinical settings; demonstration of robustness of the principal trial conclusions with respect to alternative choices of participant populations for analysis, alternative models, alternative inclusion of covariates, etc.; special subgroup analyses (e.g., especially large or small responses); and so on. Refer to the applicable section of the approved version of the SAP (in Appendix 16.1.9) for the applicable statistical/analytical issues and cite any supporting data sources for Additional Relevant Data and Relevant Participant Listings.

11.9.1. Appropriateness of Model

Address whether any checks were performed to assess whether the regression model assumptions were valid on the final data, and if any assumptions were violated, what measures were taken (e.g., a log transformation was necessary that was not anticipated in the SAP).

11.9.2. Adjustment of Covariates

Describe selection of and adjustments for demographic or baseline measurements, concomitant therapy, or any other covariates or prognostic factors. Include methods of adjustment, results of analyses, and supportive information (e.g., ANCOVA or Cox regression output) in the detailed documentation of statistical methods. If the covariates or methods used in these analyses differed from those planned in the protocol, the differences should be explained and, where possible and relevant, the results of planned analyses should also be presented. Although not part of the individual study report, comparisons of covariate adjustments and prognostic factors across individual studies may be an informative analysis in a summary of clinical efficacy data.

11.9.3. Handling of Dropouts and Missing Data

Describe the factors that may have affected dropout rates in the study (e.g., duration of the study, the nature of the disease, the efficacy and toxicity of the drug under study, and other factors that are not therapy related). Several factors should be considered and compared for the treatment groups in analyzing the effects of dropouts: the reasons for the dropouts, the time to dropout, and the proportion of dropouts among treatment groups at various timepoints.

The results of the trial should be assessed for both the subset of participants who completed the study and for the entire participant population as randomized or at least for all those with any on-study measurements.

Ignoring the participants who dropped out of the study and drawing conclusions based only on participants who completed the study can be misleading. A large number of dropouts, however, even if included in an analysis, may introduce bias, particularly if there are more early dropouts in one treatment group or the reasons for dropping out are treatment or outcome related. Although the effects of early dropouts, and sometimes even the direction of bias, can be difficult

to determine, possible effects should be explored as fully as possible. It may also be helpful to examine modeling approaches to the evaluation of such incomplete data sets.

Procedures for dealing with missing data (e.g., use of estimated or derived data) should be described. Detailed explanation should be provided as to how such estimations or derivations were done and what underlying assumptions were made.

11.9.4. Interim Analyses and Data Monitoring

The process of examining and analyzing data accumulating in a clinical trial, either formally or informally, can introduce bias and/or increase type I error. Therefore, describe in full all interim analyses, formal or informal, preplanned or ad hoc, by any study participant, Sponsor staff member, or data monitoring group, even if the treatment groups were not identified. The need for statistical adjustment because of such analyses should be addressed. Any operating instructions or procedures used for such analyses should be described. The minutes of meetings of any data monitoring group and any data reports reviewed at those meetings, particularly a meeting that led to a change in the protocol or early termination of the study, may be helpful and should be provided in Appendix 16.1.9. Data monitoring without code-breaking should also be described, even if this kind of monitoring is considered to cause no increase in type I error.

11.9.5. Multicenter Studies

A multicenter study is a single study under a common protocol, involving several sites (e.g., clinics, practices, hospitals) where the data collected are intended to be analyzed as a whole (as opposed to a post hoc decision to combine data or results from separate studies). Individual site results should be presented, however, where appropriate, e.g., when the sites have sufficient numbers of participants to make such analysis potentially valuable, the possibility of qualitative or quantitative treatment-by-site interaction should be explored. Any extreme or opposite results among sites should be noted and discussed, considering such possibilities as differences in study conduct, participant characteristics, or clinical settings. Treatment comparison should include analyses that allow for site differences with respect to response. If appropriate, demographic, baseline, and postbaseline data, as well as efficacy data, should be presented by site, even though the combined analysis is the primary one.

11.9.6. Multiple Comparison/Multiplicity

False-positive findings increase in number as the number of significance tests (number of comparisons) performed increases. If there was more than one primary endpoint (outcome variable) or more than one analysis of a particular endpoint, or if there were multiple treatment groups or subsets of the participant population being examined, the statistical analysis should reflect awareness of this and either explain the statistical adjustment used for type I error criteria or give reasons why it was considered unnecessary.

11.9.7. Use of an “Efficacy Subset” of Participants

Particular attention should be devoted to the effects of dropping participants with available data from analyses because of poor compliance, missed visits, ineligibility, or any other reason. As noted above, an analysis using all available data should be carried out for all studies intended to establish efficacy, even if it is not the analysis proposed as the primary analysis by the Applicant. In general, it is advantageous to demonstrate robustness of the principal trial conclusions with respect to alternative choices of participant populations for analysis. Any substantial differences resulting from the choice of participant population for analysis should be the focus of explicit discussion.

11.9.8. Active-Control Studies to Show Equivalence

If an active control study is intended to show equivalence (i.e., lack of a difference greater than a specified size) between the test drug/investigational product and the active control/comparator, the analysis should show the confidence interval for the comparison between the two agents for critical endpoints and the relation of that interval to the prespecified degree of inferiority that would be considered unacceptable.

11.9.9. Subgroup Analyses

If the size of the study permits, important demographic or baseline value-defined subgroups should be examined for unusually large or small responses and the results presented, e.g., comparison of effects by age, sex, or race; by severity or prognostic groups; and by history of prior treatment with a drug of the same class. If these analyses were not carried out because the study was too small, it should be noted. These analyses are not intended to “salvage” an otherwise nonsupportive study but may suggest hypotheses worth examining in other studies or be helpful in refining labeling information, participant selection, or dose selection. Where there is a prior hypothesis of a differential effect in a particular subgroup, this hypothesis and its assessment should be part of the planned statistical analysis.

11.10. Efficacy, Pharmacokinetic, Pharmacodynamic, and Biomarker Conclusions

Summarize the overall efficacy, PK, PD, and biomarker results of the study treatment. At a minimum, address all primary endpoints. Conclusions should be consistent with those in Section 13 (Conclusions), and the Conclusions portion of Section 2 (Synopsis) should be consistent with these.

12. SAFETY RESULTS

Minimize use of participant-identifiable information in the text. Inclusion of IDs and age are acceptable, but avoid specifying country and date of birth. To minimize participant-level information in the text, include cross-references to narratives and participant data listings in Appendix 16.2, as applicable.

If any critical measurements or assessments of safety outcomes were made by more than one party (e.g., both the Investigator and an expert committee may offer an opinion on whether a participant had an acute infarction), overall differences between the ratings should be shown.

Define the Safety Population, referencing the applicable table in Section 10.2, the protocol (in Appendix 16.1.1), or the SAP (in Appendix 16.1.9), as applicable. Describe other key aspects of statistical analysis of safety data, including (for example): whether safety treatment groups are different from ITT treatment groups, whether the Safety Population was used for all safety analyses or if there were additional populations (e.g., immunogenicity population, safety MRI population), and so on. Add a reference to Section 12.1.2.1 for a definition of TEAEs and for information about censoring of safety data after intercurrent events.

Give an overview of how the tables are presented (e.g., “Summary tables present safety data by treatment group and combined, unless otherwise indicated.”).

Present key safety results in tabular format, using categories that are applicable to the study. *The following table and its content is for example purposes only.* Adapt the table title and column headings as applicable to the particular study.

NOTE: See GENERAL TEXT INSTRUCTIONS FOR AUTHORS at the start of this template for guidance on manual creation of in-text tables.

Table 11: Summary of Safety Results: Placebo-Controlled and Open-Label Extension Periods

Safety Assessment	Placebo-Controlled Period (if applicable)	Open-Label Extension Period (if applicable)	Sources
Deaths	Deaths were balanced across the 2 treatment arms (xx participants [xx%] who received <study treatment> and xx participants [xx%] who received placebo) and no signals were identified from the data. <add more details, if applicable, e.g., anticipated in population due to disease or due to drug>	(no)/x death(s), and no signals were identified from the data	<list source tables, figures, etc.>
SAEs	SAEs were balanced across the 2 treatment arms (xx participants [xx%] who received <study treatment> and xx participants [xx%] who received placebo) and no signals were identified from the data. <add more details, if applicable, e.g., anticipated in population due to disease or due to drug>	The profile and incidence of the most common SAEs and <study treatment>-related SAEs was similar to the PC period.	
AEs that led to discontinuation of study treatment	Study treatment was discontinued due to AEs in xx participants (xx%) who received <study treatment> and xx participants (xx%) who received placebo.	The profile and incidence of AEs that led to discontinuation of study treatment was similar to the PC period.	

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Safety Assessment	Placebo-Controlled Period (if applicable)	Open-Label Extension Period (if applicable)	Sources
AEs	Adverse events were experienced by xx participants (xx%) who received <study treatment> and xx participants (xx%) who received placebo. Overall, a similar proportion of <study treatment>-treated and placebo-treated participants reported common AEs. <if applicable, discuss any differences> Adverse events experienced by participants with at least 2% higher incidence proportion for <study treatment> compared to placebo were AE1, AE2 etc. The majority of participants experienced AEs that were mild (<study treatment>: xx participants [xx%] and placebo: xx participants [xx%]) or moderate (<study treatment>: xx participants [xx%] and placebo: xx participants [xx%]) in intensity. Severe or very severe AEs experienced by participants with at least 2% higher incidence proportion for <study treatment> compared to placebo were AE1, AE2 etc.	The profile and incidence of the most common AEs and <study treatment>-related AEs were similar to the PC period.	
AEIs			
Other Safety Parameters	Report clinically relevant findings considered to be related to the investigational product in clinical laboratory evaluations, ECGs, vital signs, C-SSRS, and MRI.	Similar profile to the PC period	

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12.1. Deaths, Serious Adverse Events, and Other Significant Events

12.1.1. Deaths

Provide a summary analysis of deaths that occurred during the study, including the pretreatment (Screening) period and post-treatment follow-up periods, and deaths that resulted from a process that began during the study. Reference supporting tables. If no deaths were reported in the study, this should be stated. Reference participant narratives in the appropriate appendix (Appendix 16.2), as applicable.

12.1.2. Serious Adverse Events

Provide a summary analysis of SAEs (e.g., SAEs, treatment-related SAEs), referencing supporting summary tables. Include details such as the number of participants who had an SAE, the treatment group they were in, the number of SAEs that were considered related to study treatment, the number of SAEs that led to withdrawal from study. Discuss any notable differences in occurrence between treatment groups for the most common SAEs. Provide an incidence table of SAEs by preferred term (for larger studies with many SAEs). Reference the participant narratives and listings in Appendix 16.2 for more details.

If it is necessary to discuss any individual participant-level information in text, consider data presentations that maintain data meaning, remain in context, and conform to current minimum standards for de-identifying data (e.g., participant ID and age are permitted, but information such as country of origin or date of birth are not). If no SAEs were reported in the study, this should be stated. Reference the participant narratives in Appendix 16.2, as applicable.

12.1.3. Adverse Events of Special Interest

Delete any subsections that are not applicable to the study.

Provide a summary analysis of AEs of special interest as predefined in the protocol, referencing supporting summary tables. Subsections may be inserted for clarity. Present data on the incidence of any AEs that are of particular relevance to the study (e.g., infusion reactions, infections, hepatotoxicity, prolonged QT interval, etc.), and discuss any differences between groups. Insert additional headings, as needed.

If it is necessary to discuss any individual participant level information in text, consider data presentations that maintain data meaning, remain in context, and conform to current minimum standards for de-identifying data (e.g., participant ID and age are permitted, but information such as country of origin or date of birth are not). Reference the participant narratives in Appendix 16.2, as applicable.

12.1.4. Discontinuation of Study Treatment Due to Adverse Events

Briefly summarize AEs that led to discontinuation of study treatment, referencing supportive summary tables. Discuss any notable differences in occurrence between the groups for the most common events. Insert additional headings, if needed.

Reference the participant narratives in Appendix 16.2, as applicable.

12.1.5. Withdrawal From Study Due to Adverse Events

Provide a summary analysis of AEs that led to withdrawal from the study and discuss any notable differences in occurrence between the groups for the most common events. Insert additional headings, if needed. Reference supporting summary tables and any applicable appendix listings.

12.1.6. Dose Modifications Due to Adverse Events

Briefly summarize any dose modifications that occurred due to AEs (e.g., dose reduction, significant additional concomitant therapy), as applicable. Add subsections for clarification, as needed. Reference the participant narratives and listings in Appendix 16.2 for more details.

12.1.7. Malignancies

Provide a summary analysis of malignancies (e.g., the number of participants who developed a malignancy, the types of malignancies reported, the treatment group they were in, the number that were considered related to study treatment, the number that lead to withdrawal from study). Discuss any notable differences in occurrence between treatment groups for the most common malignancies. Reference the participant narratives in Appendix 16.2, as applicable.

12.1.8. Pregnancies

Provide information on the number of pregnancies in each treatment group, the impact of the pregnancies on study treatment, and the outcome of each pregnancy.

Ensure that participant data listings of all pregnancies and outcomes of pregnancies are included in the appropriate appendices, as applicable.

12.1.9. <Other Significant Events>

This section is optional; delete if not applicable to the study.

Present data for other significant events (e.g., marked laboratory abnormalities that were not AEs or SAEs but were considered clinically significant, events that were not AEs or SAEs but still led to an intervention), and discuss any notable differences in occurrence between the groups for the

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most common events. Insert subheadings, as needed. Reference the participant narratives in Appendix 16.2 for more details, if applicable.

12.2. Adverse Events

12.2.1. Overall Summary of Adverse Events

Briefly summarize the overall AE study results (cite the applicable table, as appropriate). Do not repeat the data that appear in tables. Text should instead characterize patterns, highlight differences, call attention to outliers, and point out other helpful observations to a reviewer who is going to then view the tables.

State that a participant having the same event more than once is counted only once in the incidence for that event.

Clarify whether the AEs presented are treatment-emergent AEs (TEAEs).

If so, include the definition of TEAEs (i.e., AEs that emerge or worsen on or after the first dose of study treatment; see the protocol for exact wording). Also clarify whether there was any censoring for AEs starting <X> days after treatment discontinuation, or for AEs that occurred after receiving rescue medication and/or certain concomitant medications. If a listing is provided for predose AEs and (if applicable) postcensoring AEs, include a cross-reference to those.

All AEs were classified and coded using MedDRA version <XX>.

If a participant had the same event more than once, it is counted only once in the incidence for that event.

Treatment-related AEs (defined as <include definition from protocol> were reported for <number of participants> <(x%)> treated with <study treatment> and for <number of participants> <(x%)> treated with placebo.

Summarize the type of data presented in this section (e.g., overall incidence of AEs, incidence of AEs by severity, incidence of AEs by relationship to study treatment, and incidence of infections or events associated with infections). Indicate how rows are ordered in the tables describing AEs by preferred term and/or primary system organ class.

The incidence of all AEs reported during the study was <describe similarities and/or differences between treatment groups, as applicable>.

Provide a summary analysis of AEs by system organ class and by preferred term, limiting the discussion to differences in frequency, incidence, time from administration, treatment group, and so on. Insert and reference summary tables for incidence of AEs appropriate tables and/or listings in Appendix 16.2.

Reference supporting summary tables. Discuss the total number of participants who experienced at least 1 AE, the most common AEs (cutoff determined by in the SAP or by SMT), and any differences between treatment groups.

The following suggested subsections, and/or others, can be included or omitted as applicable to the study.

12.2.2. Public Health Emergency-Related Adverse Events

Describe the incidence of AEs related to public health emergencies. If applicable, add a summary statement and cross-references to relevant in-text tables and to appendix tables or listings that contain additional data (e.g., suspected public health emergency-related adverse events by system organ class and preferred term – safety population).

12.2.3. Analysis by Severity

Indicate that the Investigator assessed the severity of each AE. Indicate the grading system used to classify AEs and version number, if applicable (e.g., NCI CTCAE Version x.x) and/or the classifications (e.g., mild, moderate, severe; or Grades 1 through 5).

Present a summary analysis of the incidence of mild, moderate, and severe AEs by treatment group and discuss any notable differences between groups, as well as any shifts from baseline to worsening grades of severity. Indicate that a participant having the same event more than once is counted only once and at the highest severity for that event.

12.2.4. Analysis by Relationship to Study Treatment

Indicate that the Investigator assessed the relationship of each AE to study treatment. Indicate the classifications (e.g., not related, unlikely, possibly, probably). Refer to section in the protocol that defines which classifications are considered “related” for the purposes of the tables.

Present a summary analysis of the incidence of AEs related to study treatment. Discuss the total number of participants who experienced at least 1 related AE, the most common (cut-off to be determined by the SMT) related AEs, any related severe/³Grade 3 AEs, and any differences between treatment groups.

12.2.5. Other AEs of Note

This section is optional; delete if not applicable to the study.

Use this section for AEs that may be reported pre-dose at screening, or for other AEs that SMEs decide are appropriate to present separately. Note that AESI and “Other Significant Adverse Events” are not presented here but in Section 12.1.1.6 and Section 12.1.1.9, respectively.

12.3. Participant Safety Narratives

Narratives for participants who died or who experienced an SAE or an AE leading to treatment discontinuation or study withdrawal are presented in [Appendix 16.2](#).

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12.4. Clinical Laboratory Results

Provide an introduction describing what will be presented in this section and how clinical laboratory evaluations are grouped.

Explain how data have been presented (e.g., shifts from baseline to low and from baseline to high). If applicable, provide definitions for “low,” “high,” and “baseline” and describe how participants are categorized (e.g., Shifts to low include participants with a normal, high, or unknown baseline value and at least 1 post-baseline value of the given test. Shifts to high include participants with a low, normal, or unknown baseline value and at least 1 post-baseline value of the given test.).

12.4.1. Hematology Results

Present hematology laboratory results per treatment group. Discuss any notable differences between treatment groups and any associated AEs. If applicable, create separate subheadings to discuss hematology results that are of particular importance to the study (e.g., platelet counts).

May reference Appendix 16.2 for narratives of participants with notable shifts outside the normal range (Study Medical Director to advise).

12.4.2. Blood Chemistry Results

Present a summary of blood chemistry results per treatment group. Discuss any notable differences between treatment groups and any associated AEs. If applicable, create separate subheadings to discuss blood chemistry results that are of particular importance to the study (e.g., liver function tests).

May reference Appendix 16.2 for narratives of participants with notable shifts outside the normal range (Study Medical Director to advise).

12.4.3. Urinalysis Results

Present a summary of urinalysis results per treatment group. Discuss any differences between treatment groups and any associated AEs. If applicable, create separate subheadings to discuss urinalysis results that are of particular importance to the study (e.g., drug metabolites in urine).

12.5. Vital Sign Measurements, Physical Examination Findings, and Other Observations Related to Safety

12.5.1. Vital Sign Measurements

Discuss incidences of clinically significant vital signs and any notable differences between treatment groups. Reference applicable tables in Section 14.3 and participant data listings in Appendix 16.2, as needed.

12.5.2. ECG Findings

This section may be deleted if not applicable to the study.

Discuss incidences of clinically significant ECG findings, including QT evaluations, and any notable differences between treatment groups. Reference applicable tables in Section 14.3 and participant data listings in Appendix 16.2, as needed.

12.5.3. Physical Examination Findings

This section may be deleted if not applicable to the study.

Any abnormal physical examination findings would be considered AEs and should be discussed under AEs. There are no longer table/listing outputs on results of physical examinations; the CRF only captures whether they were completed as planned.

Reference applicable tables in Section 14.3 and participant data listings in Appendix 16.2, as needed.

12.5.4. <Study-Specific Observations>

This section may be deleted if not applicable to the study.

Discuss any notable observations related to safety that are of particular importance in the study (e.g., slit lamp biomicroscopy, ophthalmoscopy results) and any notable differences in occurrence between the treatment groups. Present data here or in Section 14.3.

12.6. <Immunological Assessments>

This section may be deleted if not applicable to the study.

Present data for immunological assessments consistent with the SAP (e.g., cellular and humoral immunity, white blood cell phenotype counts, serum immunoglobulin concentrations, incidence of antidrug antibodies). State whether neutralizing antibodies were assessed, how antibody-positive response was determined, and whether antibody response was categorized as transient vs. persistent. Discuss any notable differences between treatment groups.

12.7. Columbia Suicide Severity Rating Scale

Suggested text:

C-SSRS results for all randomized patients are provided in Listing 16.X.X.X.

There were no C-SSRS findings of concern. No suicidal ideation, suicidal behavior or non-serious self-injurious behavior emerged in any patient during the study (Table 14.X.X).

12.8. <Study-Specific Subgroups>

This section may be deleted if not applicable to the study.

Discuss safety results relevant to any study-specific subgroups (e.g., analyses by age group, sex, weight, prior history of disease).

12.9. Safety Conclusions

Summarize the overall safety of the study treatment. At a minimum, address all major safety objectives. Conclusions should be consistent with Section 13 (Conclusions), and the Conclusions portion of Section 2 (Synopsis) should also be consistent with these.

13. CONCLUSIONS

Use this section to describe what was learned from the study, focusing on the primary and secondary objectives. The conclusions presented here should also be included in the Conclusions portion of Section 2 (Synopsis), as appropriate for the study.

Summarize the overall results of the study, as follows:

- Summarize the overall safety of the study treatment, as applicable.
- Comment on any unexpected findings and on their significance.
- Discuss any problems or difficulties that were encountered (e.g., inconsistencies between related measures) or any unanticipated study limitations, as applicable.
- Include a general statement about where this study fits into the clinical development plan and whether the results support continued study.
- Reference relevant tables, figures, and sections if necessary.

Do not include the following:

- Do not restate the results already presented in the CSR.
- Do not introduce any new results. Conclusions should be supported by data already presented in the report.
- Do not recap or summarize study design or participant demography.
- Do not discuss results of other clinical trials of the same drug.
- Do not present detailed interpretations about the general benefit-risk of the study treatment.

14. TABLES, FIGURES, AND GRAPHS REFERENCED, BUT NOT INCLUDED IN THE TEXT

Only aggregate data should be presented in this section. All listings of participant-level data should be included in Appendix 16.2.

NOTE: If there are more than approximately 50 end-of-text tables and figures for Section 14.1 through 14.3, these outputs will be added as separate groupings of PDFs external to the CSR body, each with its own table of contents (provided by Stats Programming). *This approach can also be applied for fewer than 50 TLGs if it will result in improved usability, organization, and/or access to the TLGs.* Consult the CSR sub-process owner and Document Quality for guidance on how to cite these TLGs in the CSR body.

14.1. Study Participants

Include tables and figures referenced in the corresponding section of the report but not included in the text.

14.2. Efficacy, Pharmacokinetic, Pharmacodynamic, and Biomarker Data

Include tables and figures in the corresponding section(s) of the report that are referenced but not included in the text of the report.

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14.3. Safety Data

Include tables and figures in the corresponding section(s) of the report that are referenced but not included in the text of the report.

14.3.1. Narratives of Deaths, Serious Adverse Events, and Other Significant Adverse Events

Narratives for deaths, SAEs, and AEs that resulted in discontinuation of study treatment or withdrawal from the study are presented in [Appendix 16.2.7](#).

14.3.2. Summary of Adverse Events

14.3.3. Summary of Clinical Laboratory Results

14.4. Additional Information on Study Design and Implementation

In tabular format, provide information on vendors used in the study. Provide the name of the company/individual, their address, and a general description of their responsibilities (e.g., data management, study clinical supply management, site monitoring, statistical analysis, and medical writing).

Provide, in tabular format, information on vendors used in the study. The vendors presented should have had a significant role in study conduct; data handling, analysis, and reporting, (e.g., CROs, central laboratories, centralized randomization services, electronic data capture services, MRI reading centers, contract/FSP Medical Writers); and safety reporting. (Safety reporting vendors are defined as those that report the individual case safety reports [ICSRs] to Investigators and health authorities.). Do not list any vendors that were subcontracted through the main CRO. Be sure to review the vendor list with SMT members to identify errors and/or inconsistencies.

Table 12: Vendors That Participated in Study <insert study number>

Vendor Name	Vendor Address	Responsibilities
<vendor name, e.g., IQVIA>	<Street Address Town, State, Country, Postal Code>	<Services provided, e.g., Data management and site monitoring>
<vendor name, e.g., Synchrogenix>	<Street Address Town, State, Country, Postal Code>	<Services provided, e.g., Medical Writing services>
...		

15. REFERENCE LIST

Provide a list of all publications referenced in the text. References should be inserted using EndNote.

Key publications are included in [Appendix 16.1.12](#). All remaining references are available upon request.